

Question 1 : Explain how you would create a plan of rehabilitation with a patient addicted to opioids using a biopsychosocial framework. (Make sure to incorporate biological, psychological and social aspects into your plan).

My rehab plan (if money and access were not factors!) for an individual addicted to opioids would begin with addressing the immediate biological aspects of opioid abuse and withdrawal in an inpatient setting; a medication such as methadone or suboxone could be administered to initiate the process, while other biological factors such physical withdrawal symptoms could be monitored and treated if needed (like giving IV fluids, for example, if vomiting is severe). I would also develop a nutritional plan with them to replenish nutrients lost during active addiction and the process of withdrawal. Most importantly, a full physical exam should be done to address any underlying chronic pain issues that may have initiated the addiction to opioids and also to establish what kind of physical damage has been incurred during active addiction (dental decay, exposure to MRSA, etc).

Once the patient is stable, psychological factors can be addressed outpatient; support via a therapist can introduce a patient to mindfulness training, which can help with awareness and acceptance of one's situation or help cope with opioid use triggers. CBT can also help a patient modify behaviors to improve coping skills and reduce harmful or impulsive behaviors that may lead to drug use. As in the biological aspect, it would be important to address any underlying causes of chronic pain, including emotional pain, so ongoing therapy and potentially psychiatric intervention (i.e. SSRIs if an underlying cause of abuse is depression) may help curb any future urges. Support groups such as NA (Narcotics Anonymous) also provide ongoing psychological support as well as social support, including direct peer support with sponsors and recovered community members.

Social factors and support are especially vital to maintaining a rehab plan long-term and promoting adherence; in addition to support groups, engaging with peers and family who promote healthy behaviors is important, and eliminating environments and people who may trigger or encourage drug use (or make drugs available) is equally important. I would have a social worker connect with the patient and support job-seeking activities, facilitate access to social services that may reduce life stressors and improve recovery, provide resources for drug use and abuse education, and generally assist in building structure and providing support in improving one's socioeconomic circumstances. If the patient still has family willing to provide a strong support system, I would provide them with resources to support them as well, such as information about Nar-anon (or similar support groups for those affected by someone's drug abuse), individual therapy, books, and community resources. Additionally, I would consider where this patient is from (or lives), identify cultural or social proclivities toward opioid use, and use that information to tailor any specific aspects of treatment to addressing stigma or ambivalence toward drug use or treatment.